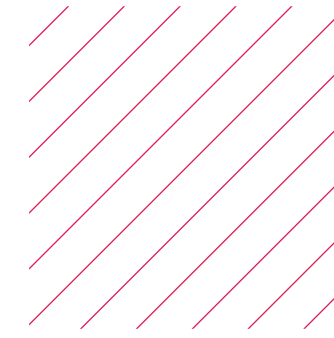




CI INSTITUTE OF NURSING

CERTIFIED NURSE ASSISTANT
TRAINING PROGRAM



Module 9: Patient Care Procedures

CI INSTITUTE

MODULE 9:

Patient Care

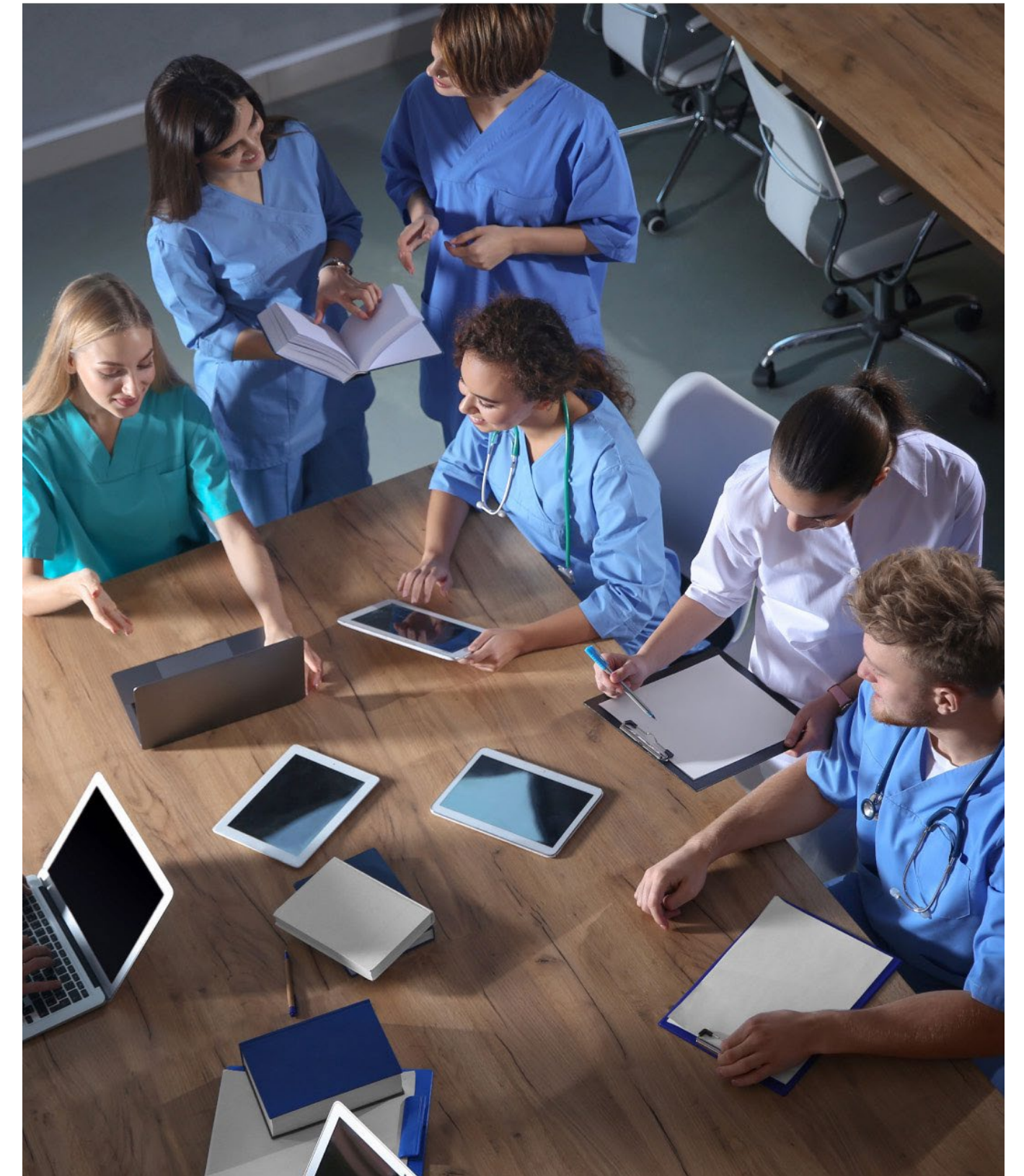
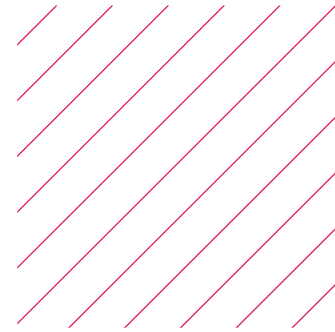
Procedures

Minimum Number of Theory Hours:

7

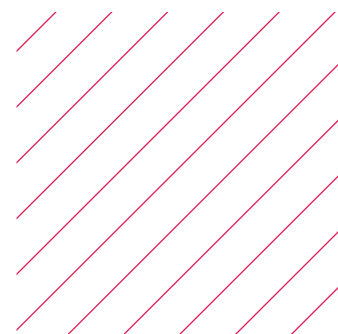
Recommended Clinical Hours:

20



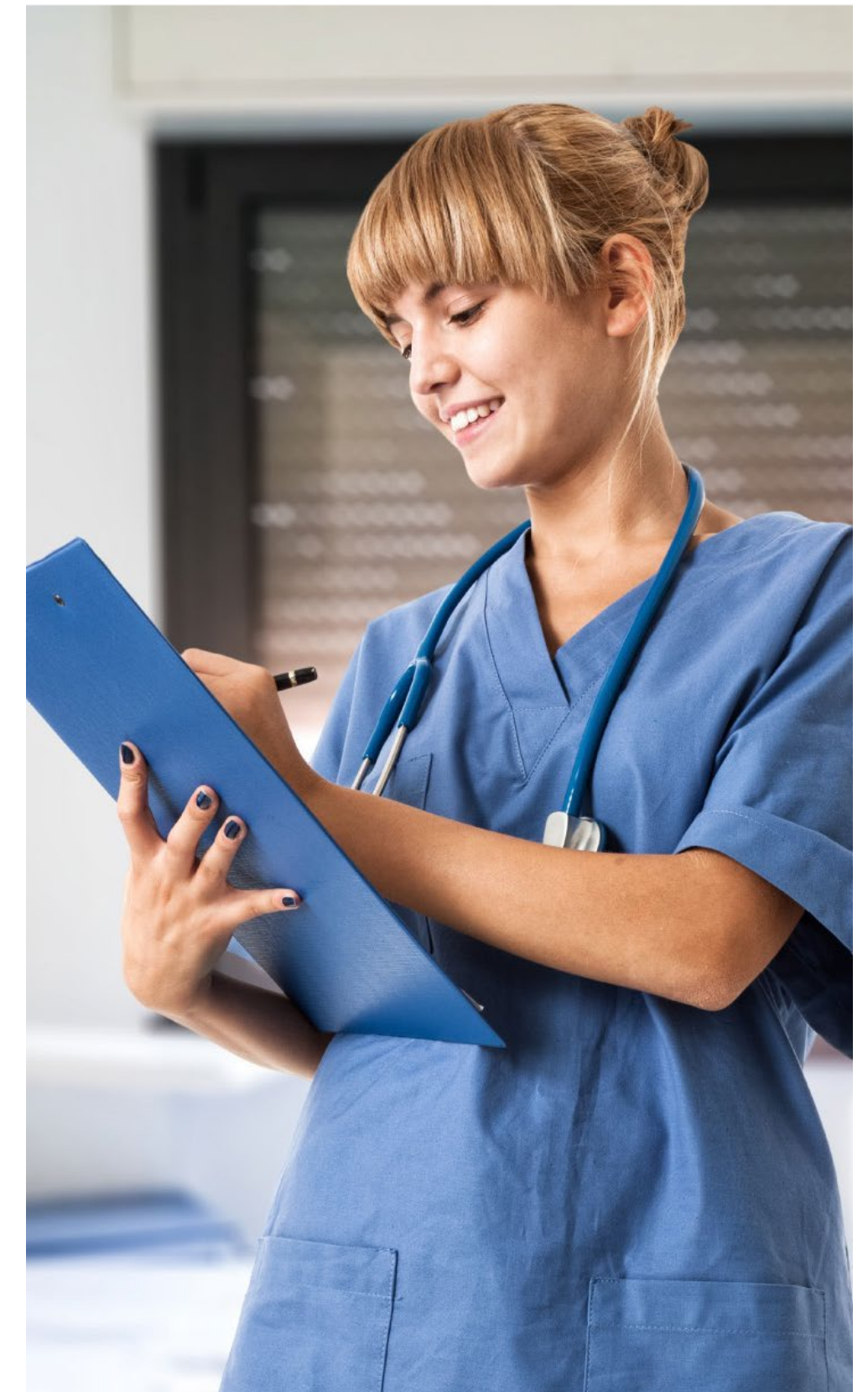
STATEMENT OF PURPOSE

The purpose of this unit is to provide learning experiences that will prepare the Nurse Assistant to safely carry out procedures that support the patient/resident in meeting physical care needs that cannot be performed independently.



TERMINOLOGY:

- | | | |
|---|----------------------------|----------------------|
| 1. Admission | 2. Anti-embolic stockings | 3. Bandage |
| 4. Binders | 5. Clean catch | 6. Defecate |
| 7. Discharge | 8. Draw sheet | 9. Edema |
| 10. Elastic bandage | 11. Electric bed | 12. Evacuation |
| 13. Excoriated | 14. Expectorate | 15. Fanfold |
| 16. Fluid | 17. Gastrostomy tube | 18. Hives |
| 19. Intake | 20. Integumentary system | 21. Intravenous (IV) |
| 22. Lesions | 23. Manual bed | 24. Mitered corner |
| 25. Mucous | 26. Nasogastric tube (NGT) | 27. Non-prescription |
| 28. Occupied bed | 29. Ointment | 30. Output |
| 31. Percutaneous Endoscopic Gastrostomy (PEG) | 34. Reverse Trendelenburg | 32. Pruritus |
| 33. Reflux | 37. Side rails | 35. Scaly |
| 36. Semi-Fowler's position | 40. Transfer | 38. Specimen |
| 39. Suppository | | 41. Trendelenburg |
| 42. Unoccupied bed | | |



Patient, resident, and client are synonymous terms referring to the person receiving care



PERFORMANCE STANDARDS

Upon completion of seven (7) hours of class plus assignments and twenty (20) hours of clinical experience, the learner will be able to:

1. Define key terminology COLLECTION of SPECIMENS
2. Describe the Nurse Assistant's role in collecting specimens (sputum, urine, and/or stool) BED CARE
3. Discuss procedures for bed making and maintaining proper body mechanics
4. Discuss types of beds and bed positions
5. Discuss ways to maintain the patient's/resident's environment

BOWEL CARE

6. Describe the Nurse Assistant's role in administering an enema
7. Describe the Nurse Assistant's role in giving a suppository TUBES
8. List the types and uses of gastrointestinal (GI) tubes
9. Describe major nursing care activities for patients/residents with feeding tube (nasogastric or gastrostomy)
10. Describe nursing care activities for a patient/resident receiving intravenous (I.V.) therapy INTAKE AND OUTPUT

11. Describe the Nursing Assistant's role in assisting the patient/resident to maintain fluid balance
12. Describe the purpose and procedure for measuring the amount of fluid taken in and fluids excreted by the patient/resident BANDAGES AND

DRESSINGS

13. Discuss the Nursing's Assistant role in the use of bandages, binders and dressings
14. Identify the Nurse Assistant's role in the care of patient's/resident's skin conditions and the use of non-legend topical ointments, lotions, or powders.
15. Identify the Nurse Assistant's role in the care of patients/residents' skin conditions and the use of non-prescription ointments, lotions, or powders

ADMISSION, TRANSFER and DISCHARGE

16. Explain the role of the Nurse Assistant in the admission of a patient/resident to the facility
17. Explain the role of the Nurse Assistant in transferring a patient/resident from one area to another within the facility
18. Explain the role of the Nurse Assistant in the discharge of a patient/resident



/// DEFINE KEY TERMINOLOGY

- A. Review the terms listed in the terminology section
- B. Spell the listed terms accurately
- C. Pronounce the terms correctly
- D. Use the terms in their proper context



Describe the Nurse Assistant's role in collecting specimens (sputum, urine, and/or stool).

A. Sputum Specimens

1. Purpose

- a. Respiratory disorders cause the secretion of mucus from lungs, bronchi and trachea
- b. Mucous secretion is called sputum (not related to saliva)
- c. Expecterated from upper airways
- d. Sputum is studied for blood, microorganisms, and abnormal cells

2. Assisting the patient/resident in raising sputum for a specimen

- a. Secretions are more easily coughed up after the patient/resident wakes up in early a.m.
- b. Allow the patient/resident to rinse their mouth with water, as this reduces the amount of saliva in the mouth and removes food particles
- c. Do not use mouthwash, as this may destroy some of the organisms
- d. Coughing up sputum may be embarrassing to the patient/resident, and may nauseate others nearby
- e. The specimen itself may be perceived as unpleasant Keep container covered and place in bag
- f. Privacy is important
- g. Use standard precautions

3. Specimen needs to be labeled

- a. Patients/residents full name
- b. Room and bed number
- c. Time and date the specimen was collected

4. Observations

- a. Color
- b. Odor
- c. Consistency
- d. Blood

5. Document specimen obtained and were taken

Describe the Nurse Assistant's role in collecting specimens (sputum, urine, and/or stool).

B. Urine Specimens

1. Purpose

- a. Urine is collected for a laboratory examination or testing in the unit to help the physician diagnose a problem or evaluate treatment

2. Methods for collecting urine specimens

- a. Mid-stream
- b. Clean catheter urine specimen
- c. 24-hour urine specimen

3. Rules to follow in collecting urine specimen

- a. Wash hands before and after collecting a specimen
- b. Use standard precautions
- c. Use the correct and clean container for each specimen
- d. Label the container accurately
 - 1) Patient's name
 - 2) Room and bed number
 - 3) Date and time specimen was collected

- e. Collect specimen directly into container at time specified

- f. Do not touch the inside of the container or lid

- g. Ask the patient/resident not to have a bowel movement while the specimen is being collected

- h. Ask the patient/resident to place toilet tissue in the toilet or wastebasket

- i. Take the specimen and the requisition slip to the designated lab pick-up station

- j. Document that the specimen was obtained and where it was taken in the patient/resident record

4. Observations

- a. Difficulty obtaining specimen

- b. Color, clarity and odor

- c. Particles of discomfort and/or urgency

Describe the Nurse Assistant's role in collecting specimens (sputum, urine, and/or stool).

C. Stool Specimens

1. Stool is collected for a laboratory test to check for the presence of blood, fat, micro-organisms, worms (parasites), and any abnormal contents
2. General rules for collection
 - a. Maintain patient's/resident's privacy
 - b. Use standard precautions
 - c. Use clean container
 - d. Give the patient/resident clear instruction on how to defecate for the specimen; must not contaminate with urine or toilet tissue
 - e. Label the container accurately
 - 1) Patient's/resident's name
 - 2) Room and bed number
 - 3) Date
 - 4) Time specimen was collected
 - f. Clarify if the specimen must be kept at room temperature, or refrigerated (check on specific lab test)
 - g. Take specimen and requisition slip to the designated area
3. Observations
 - a. Difficulty in obtaining specimen
 - b. Color, amount, consistency, and were taken
 - c. Complaints of pain or discomfort
4. Document specimen obtained and were taken

Discuss procedures for bed making and maintaining proper body mechanics.

A. Bed making is an important part of the Nurse Assistant role

1. A clean, dry, neat bed makes the patient/resident more comfortable
2. The body exerts uneven points of pressure over bony prominences (elbow, sacrum), etc., when against the mattress for extended periods. Use a mattress pad, if ordered
3. Bed linen must be wrinkle-free to prevent it from becoming an irritant to skin
4. The state of mind of the patient/resident is affected by the condition of the bed. To create and maintain a positive attitude, the bed should be kept
 - a. In good repair
 - b. Clean bedframe by damp dusting daily
 - c. Well-made, wrinkle-free
 - d. Attractive with use of bedspread
5. Keep call bell in place, clipped to linen
6. Keep the patient/resident reality-oriented by use of patient's/resident's personal pillow, afghan, etc. (familiar objects)

Discuss procedures for bed making and maintaining proper body mechanics.

B. Bed linen can act as an irritant to patient/resident skin

1. Use sheets to separate the blanket from the patient's/resident's skin, thus preventing blanket fibers from causing irritation
2. Keep bottom linens tucked in and wrinkle-free
3. Cover plastic draw sheet with cotton draw sheet
4. Straighten and tighten loose sheets, blankets and bedspreads whenever necessary
5. Strong laundry soaps may cause skin irritation; observe for patient/resident skin problems and report

C. Aseptic technique is important when handling bed linen; follow standard precautions

1. Wash hands. Germs spread by cross-contamination
2. Hold and carry linen away from uniform
 - a. Dirty linen is dirtier than the uniform and could transfer micro-organisms to the uniform
 - b. Clean linen is cleaner than the uniform
3. Shaking linens or fluffing them in the air is avoided during bed making since this practice will spread dust and germs
4. Keep linen off the floor or off the overbed table, as soiled linen can transfer germs to these areas
5. Soiled linen should be immediately placed in a linen hamper or rolled tightly and tucked at the foot of the bed between the mattress and frame until it can be carried to the linen hamper
6. Soiled-linen hampers should be kept covered to prevent spread of infection and odors

Discuss procedures for bed making and maintaining proper body mechanics.

D. Body Mechanics

1. Proper body mechanics is required to make a bed to prevent injury and fatigue
2. Know your limitations. Don't try to lift, turn, or move a patient alone if you have any doubts about your ability to do so
3. Get close to sides of bed; don't make beds from the head of bed or foot positions
4. Keep back straight and knees bent
5. Keep feet apart to give a broader support base
6. Move feet to turn in the direction wanted and avoid twisting your back
7. Face in the direction you are working at the side of the bed
8. Raise bed to comfortable height
9. Make one side of bed before beginning other side

Discuss types of beds and bed positions

A. Functions and structures of a hospital bed

1. Manually operated beds have hand cranks at the foot of the bed which raise or lower the head, foot, or total bed. Cranks are to be kept down when not in use
2. Electric beds have electric controls located on the side or foot of the bed for the patient/resident and staff use
3. Side rails
 - a. Uses
 - 1) Prevent patient/resident from falling out of bed
 - 2) Provide security
 - 3) Give patient/resident support to hold or grasp when moving or turning while in bed
 - b. Regulations regarding use
 - 1) Considered restraints
 - 2) Must have consent to use
 - 3) Need for use must be noted in patient's/resident's record and care plan
 - 4) Patient/resident must be checked frequently if side rails are ordered

Discuss types of beds and bed positions

B. Bed Positions

1. High position encourages the staff to use good body mechanics when giving care to patients/residents, moving patients/residents to stretchers, or when making the bed
2. Low position is used to encourage the ambulatory patient/resident to get in and out of bed with ease and safety
3. Fowler's position (backrest raised 45 degrees to 90 degrees) is used for patient/resident comfort to eat meals and to breathe more easily in certain respiratory and cardiac conditions
4. Semi-Fowler's position (backrest 45 degrees with knees raised 15 degrees) is used for comfort and to keep the patient/resident from sliding down in bed
5. Trendelenburg position is when the head of the bed is lowered, and the foot of the bed is raised. This position requires a doctor's order
6. Reverse Trendelenburg position is when the head of the bed is raised, and the foot of the bed is lowered. This position requires a doctor's order

Discuss ways to maintain the patient's/resident's environment.

A. Patients/residents spend a lot of time in their rooms

1. Rooms should be comfortable. Control temperature, ventilation, noise, light, and odor
2. Rooms should be safe
 - a. Maintain furniture and equipment for communicating in a safe and effective manner
 - b. Patient's/resident's rights to privacy should be considered when furnishing a room. Allow the patient/resident choices and selection of own furniture from home to the extent of facility policies
 - c. Make sure bed wheels are locked when giving care or transferring patients/residents

B. General rules for maintaining the patient's/resident's unit

1. Make sure patient/resident can reach bedside stand and overbed table
2. Arrange personal belongings the way the patient/resident prefers, with patient/resident safety in mind and within easy reach
3. Keep call bell within patient's/resident's reach
4. Make sure patient/resident can reach telephone, television controls, and light controls
5. Provide the patient/resident with tissues, toilet paper and a waste container
6. Adjust lighting and temperature for patient's/resident's comfort
7. Respect patient's/resident's belongings
8. Keep environment clean
9. Straighten bed linens as needed

Describe the Nurse Assistant role in administering an enema.

A. An enema is the introduction of fluids into the rectum and lower colon; ordered by a doctor

B. The purpose of an enema is to

1. Stimulate a bowel movement
2. Relieve constipation or fecal impaction
3. Cleanse bowel of feces prior to surgery and diagnostic procedures
4. Remove flatus (gas)

C. Types of enemas

1. Cleansing enemas to remove feces from colon and rectum
 - a. Tap water
 - b. Soap suds
2. Oil retention enema; given for constipation or fecal impaction for lubrication of feces
3. Commercial mixtures; given for constipation (i.e., Fleet's)

Describe the Nurse Assistant role in administering an enema.

D. General rules to follow when giving an enema

1. Before Nursing Assistant administers enema, make sure:
 - a. State governing board allows Nursing Assistants to give enemas
 - b. Procedure is in job description
 - c. Have obtained the necessary education and training
 - d. There is a nurse available to supervise the procedure
2. Temperature of solution should be 105 degrees F.
3. Amount of solution is 500-1000 ml for adults
4. Patient/resident positioned in left Sim's position
5. Height of enema bag is no more than 18" above the mattress
6. Depth of tube insertion is about 2" to 4" into rectum and tube must be lubricated
7. Administer solution slowly over ten to fifteen minutes (Solution is usually held in rectum for a variable length of time until urge to defecate occurs)
8. Hold enema tube in place while administering
9. Make sure toilet facility is nearby and available
10. Observe the results of the enema
11. Use standard precautions

Describe the Nurse Assistant's role in giving a suppository.

A. A rectal suppository is used to

1. Stimulate to empty the bowel
2. Lubricate the stool to ease evacuation

B. Nurse Assistant role

1. Facility policy dictates if the Nurse Assistant may insert a suppository
2. General rules to follow when inserting a suppository
 - a. Nurse Assistants may not administer medicated suppositories
 - b. Identify the patient/resident by checking the arm band
 - c. Remove wrapper from the suppository (if wrapped)
 - d. Place suppository 1 to 1-1/2 inches past anal sphincter using gloved hand and index finger
 - e. Instruct the patient/resident to hold the suppository in the rectum as long as possible
 - f. Observe for results of the suppository
 - g. Report the results to the licensed nurse

List types and uses of gastrointestinal (GI) tubes.

A. Nasogastric tube

1. Inserted through nose into stomach or intestine to
 - a. Drain the GI tract by means of suction to prevent post- operative vomiting, obstruction, or gas formation
 - b. Diagnose a disease
 - c. Wash out stomach content
 - d. Provide a route for feeding one who is unable to take food by mouth

B. Gastrostomy tube/PEG

1. Surgically inserted through abdominal wall into the stomach
2. Purpose of tube is for feeding the patient/resident

Describe major nursing care activities for patients/residents with feeding tubes (nasogastric or gastrostomy).

A. Care of patient/resident with nasogastric tube

1. Give frequent oral hygiene and keep the lips and mouth moist. The mouth becomes very dry and may have a bad taste
 2. Nose and nostrils need to be cleaned frequently
 3. Increase freedom of movement by securing tubing with clamp or tape to the patient's/resident's clothing to permit maximum activity
 4. Check to see that the patient/resident does not sit or lie on tubing (tubing must be free of kinks)
 5. Check to see if the suction machine is operating satisfactorily and reporting at once if it is not working (if suction is ordered)
 6. Permitting the patient/resident (if allowed) to suck on ice chips, throat lozenges, or hard candy to keep throat slightly moist.
- Patient/resident is usually NPO (nothing by mouth) Positioning the patient/resident with head of bed elevated at 45 degrees during the feeding and for 30-60 minutes afterward, and then 30 degrees after the feeding

Describe major nursing care activities for patients/residents with feeding tubes (nasogastric or gastrostomy).

B. Care of patient/resident with gastrostomy tube

1. Give frequent oral hygiene and keep the lips and mouth moist The mouth becomes very dry and may have a bad taste
2. Increase freedom of movement by securing the gastrostomy tube with clamp or tape to the patient's/resident's clothing to permit maximum activity
3. Check to see that the patient/resident does not sit or lie on tubing (tubing must be free of kinks)
4. Permit the patient/resident (if allowed) to suck on ice chips, throat lozenges, or hard candy to keep throat, as well as tube, slightly moist
5. Position the patient/resident with head of bed elevated at all times 20 degrees to 30 degrees, to prevent reflux
6. Removing dressing from G-tube; clean and dry area; replace according to care plan
7. Reporting any unusual conditions observed during procedure
 - a. Same as nasogastric tube
 - b. Redness, swelling, drainage, odor or pain at the ostomy site

Describe major nursing care activities for patients/residents with feeding tubes (nasogastric or gastrostomy).

C. Nursing care is provided to maintain a patient's/resident's mental and emotional comfort by:

1. Keeping the environment clean, tidy, and well-ventilated, as the patient/resident is often very sensitive to odors which can cause nausea and vomiting
2. Answering call lights promptly
3. Checking frequently and giving emotional support
4. Giving an extra back rub
5. Straightening or changing bed linen prn
6. Asking patient/resident to express concerns about the tube
7. Encouraging the patient/resident to be up, dress in day clothes, and join in activities as tolerated
8. Assist patient/resident to attend family and group activities

Describe major nursing care activities for patients/residents with feeding tubes (nasogastric or gastrostomy).

D. Observe, report, and record routine care and any unusual events

1. Nausea, vomiting, diarrhea
2. Discomfort
3. Distended abdomen
4. Coughing
5. Care of indigestion or heartburn
6. Elevated temperature
7. Signs and symptoms of respiratory distress
8. Increased pulse rate
9. Care of flatulence

Describe nursing care activities for a patient/resident receiving intravenous (I.V.) therapy.

A. Reasons for using I.V. therapy

1. I.V. therapy provides the body with needed elements that can't be given as rapidly or efficiently by other means
2. I.V. may contain
 - a. Blood, plasma
 - b. Nutritional requirements for water, salt, sugar, etc.
 - c. Medications
3. Rate of I.V. flow is often controlled by an infusion pump

B. Nursing Assistant responsibilities include observing for flow from I.V.

1. Keeping tubing free of twisting or kinking
2. Observing position of tubing and condition of injection site for any infiltration
 - a. An infiltrated I.V. is one in which the needle has come out of the vein and the I.V. leaks into the tissue, causing swelling
 - b. Report this condition immediately to the charge nurse
3. Checking restraints or soft protective devices to be sure that they are not blocking the vein. Follow your agency's policy regarding restraints and soft protective measure

Describe nursing care activities for a patient/resident receiving intravenous (I.V.) therapy.

C. Nursing Assistant responsibilities

1. Maintain a patient's/resident's physical comfort
2. Bathe the patient/resident according to daily routine
 - a. Wash gently around the area where needle is inserted
 - b. Do not loosen the tape that holds needle in place
 - c. When drying, do not rub over area, pat gently to avoid dislodging the needle
3. Assist the patient/resident with eating by cutting food, preparing liquids, and arranging utensils conveniently
4. Assist the patient/resident with feeding as little as possible to encourage self-care
5. Assist the patient/resident to ambulate
 - a. Provide a portable I.V. stand
 - b. Assist out of bed
 - c. Observe closely for weakness
 - d. Support the I.V. arm to ensure continuous flow; a sling may be used to rest the arm
 - e. Patients/residents may grasp the I.V. pole for support (with I.V. hand). This provides support for the arm and lets them move at their own pace, leaving other hand free for balance by holding onto railings

Describe the Nursing Assistant role in assisting the patient/resident to maintain fluid balance.

A. Importance of maintaining fluid balance

1. Next to oxygen, water is the most important physical need
2. Death can result from taking in inadequate fluids, from losing too much fluid or too much fluid intake
 - a. Water enters body through food and fluid
 - b. Water is lost through urine, sweat, feces, and lungs
3. Balance between amount of fluid taken in and the amount of fluid lost is necessary to maintain health
 - a. Amount of fluid taken in and the amount of fluid lost must be equal
 - b. An adult needs about 2000 ml of fluid a day
 - c. Edema; fluid intake exceeds fluid output, tissues swell with water
 - d. Dehydration; fluid output exceeds fluid intake, decrease in the amount of fluid in tissues
4. Patients/residents depend on nursing personnel to meet part or all of their food and fluid needs

Describe the Nursing Assistant role in assisting the patient/resident to maintain fluid balance.

B. Encouraging increase in fluid intake

1. Forcing Fluids

- a. When physician orders “force fluids” it means to have the patient/resident drink an increased amount of fluid
- b. May order specific amount of fluid for 24-hour period
- c. Maintains fluid balance
- d. May be for general or specific amount of fluid

2. Nurse Assistant’s responsibility to encourage fluid intake

- a. Keep record of amount taken in
- b. Provide variety of fluids
- c. Place within patient’s/resident’s reach
- d. Offer fluids frequently to patients/residents who cannot feed themselves

Describe the Nursing Assistant role in assisting the patient/resident to maintain fluid balance.

C. Restricting fluid intake

1. When physician orders “restrict fluids” it means fluids are restricted to a specific amount
2. Nurse Assistant’s responsibility
 - a. A sign posted above the bed
 - b. Water is offered in small amounts
 - c. Keep water pitcher out of sight, or removed from room
 - d. Keep accurate intake and output record
 - e. Be aware of shift fluid requirements
 - f. Provide patient/resident with frequent oral hygiene
 - g. Explain to the patient/resident and family the reason for limiting fluid, removing water pitcher, etc.

Describe the Nursing Assistant role in assisting the patient/resident to maintain fluid balance.

D. Nothing by mouth (NPO)

1. Ordered

- a. Before and after surgery
- b. Before certain lab tests and x-rays
- c. In the treatment of some illnesses

2. Nurse Assistant's responsibility for NPO patient/resident

- a. NPO sign above bed
- b. Remove water pitcher and glass
- c. Offer frequent oral hygiene
- d. No swallowing of any fluid

Describe the purpose and procedure for measuring the amount of fluid taken in and fluids excreted by the patient/resident

A. Purpose of “intake and output” (I & O) measurement

1. The doctor or nurse may want to keep track of a patient’s/resident’s fluid intake and output to evaluate
 - a. Fluid balance
 - b. Kidney function
 - c. Response to medical treatment
2. Accurate measurement is needed and documented on the I & O record

B. Measurement of patient’s/resident’s intake

1. Measured in milliliters (ml)
2. Determine the fluid capacity of bowls, dishes, cups, pitchers, glasses, and other containers used to serve fluids
3. Count as intake
 - a. Water, milk, coffee, juice, soup, etc.
 - b. All food in liquid form when eaten or those that later revert to liquid, i.e., Jell-O, ice cream
4. A conversion table is provided on the intake and output record used to chart intake
5. A container called a graduated cylinder is used to measure fluid

Describe the purpose and procedure for measuring the amount of fluid taken in and fluids excreted by the patient/resident

C. Measurement of patient's/resident's output

1. Measure all fluids excreted by patient/resident done in ml.
2. All liquid output will be measured including urine, vomitus, liquid stool, drains and suction
3. Plastic urinals and emesis basins may be calibrated
4. Use standard precautions when measuring output

D. Recording intake and output

1. I & O record at bedside, document amounts when fluid is taken in or excreted
2. Amounts are totaled at end of shift and entered in the patient's record
3. Other special forms as required by facility

E. Reporting intake and output:

1. Report any unusual occurrences
2. Refusing to drink fluid, special fluid likes or dislikes, and blood in urine